

HOUSE BILL 372

By Hulsey

AN ACT to amend Tennessee Code Annotated, Title 63;
Title 68 and Title 71, relative to TennCare.

BE IT ENACTED BY THE GENERAL ASSEMBLY OF THE STATE OF TENNESSEE:

SECTION 1. Tennessee Code Annotated, Title 71, Chapter 5, Part 1, is amended by adding the following as a new section:

71-5-177.

(a) This section is known and may be cited as the "Tennessee Medicaid Modernization and Access Act of 2025."

(b) The general assembly finds that:

(1) The CMS 2024 final rule provides updated guidance on medicaid reimbursements and specifically addresses the need for competitive reimbursement rates for key services;

(2) TennCare's current medicaid fee schedule does not consistently align with the medicare fee schedule or average commercial rates, which leads to limited access for medicaid beneficiaries and inadequate compensation for healthcare providers; and

(3) Modernizing TennCare reimbursements for key services in alignment with the medicare fee schedule and the CMS 2024 final rule will improve healthcare access and outcomes, particularly in rural and underserved areas, support equitable healthcare provider compensation, expand patient access, and improve quality outcomes for medicaid beneficiaries in this state.

(c) As used in this section:

(1) "Average commercial rate" means the average commercial reimbursement rate in this state as benchmarked against the state insurance plan;

(2) "CMS" means the federal centers for medicare and medicaid services;

(3) "CMS 2024 final rule" means the May 2024 guidelines set by CMS updating medicaid reimbursement standards for key services;

(4) "Key services" means obstetrics-gynecology (OB/GYN), primary care, outpatient mental health, and substance use disorder (SUD) services;

(5) "Medicare fee schedule" means the federal payment schedule for services provided to medicare beneficiaries, as set by CMS; and

(6) "State insurance plan" means the group health insurance plan for state employees created under title 8, chapter 27.

(d)

(1) TennCare reimbursements for key services in calendar year 2025 and subsequent years must be updated to match the medicare fee schedule or the average commercial rate, whichever is higher. This subdivision (d)(1) may be implemented through phasing in for specific key services, if necessary.

(2) The department of health, in consultation with the bureau of TennCare, shall conduct an annual review to ensure TennCare reimbursement rates align with changes in the medicare fee schedule and average commercial rates and with the CMS 2024 final rule.

(3) A healthcare provider of key services that is entitled to receive TennCare reimbursement who alleges a delay in payment or receives an erroneous level of reimbursement pursuant to this subsection (d), may, in

accordance with rules promulgated by the commissioner of health, submit a written request for an administrative hearing, and the decision made after the hearing is final.

(e)

(1) Healthcare providers receiving increases under this section may receive additional incentive payments based on metrics for quality of care and improved patient access, particularly in rural and underserved areas.

(2) The department of health shall consult the bureau of TennCare to establish and enforce these quality and access metrics.

(f) The bureau of TennCare shall submit a request to CMS to modify the state medicaid plan, as necessary, to implement this section.

(g)

(1) The department of health and bureau of TennCare shall actively seek and apply for federal, private, or other available funds, and actively direct available state funds, to support reimbursement adjustments under this section, with particular attention to key services.

(2) It is the legislative intent that funds be annually appropriated in the general appropriations act to cover administrative costs to implement this section, including the costs of benchmarking, updating annual fees pursuant to subsection (d), and overseeing the incentive program pursuant to subsection (e).

(h) Beginning February 1, 2026, and no later than February 1 of each subsequent year, the department of health and the bureau of TennCare shall submit an annual joint report to the health and welfare committee of the senate and the committee of the house of representatives having jurisdiction over insurance, that details fiscal

impacts, provider participation rates, access improvements, and outcome metrics for key services impacted by this section.

(i) The bureau of TennCare and the department of health are authorized, as necessary, to promulgate rules to effectuate this section. Rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

SECTION 2. This act is not an appropriation of funds, and funds must not be obligated or expended pursuant to this act unless the funds are specifically appropriated by the general appropriations act.

SECTION 3. This act takes effect upon becoming a law, the public welfare requiring it, and unless otherwise prohibited by the United States Constitution or the Tennessee Constitution, this act applies to all TennCare reimbursements for key services occurring on or after January 1, 2025.